

COMMON SKIN DISORDERS TESTED ON NCLEX-RN

1. PSORIASIS

Chronic autoimmune (body attacking itself) skin condition causing rapid skin cell turnover.

CHARACTERISTICS:

- Thick, red plaques with silvery-white scales
- Common on elbows, knees, scalp
- Not contagious

TREATMENT:

- Topical corticosteroids
- Vitamin D analogs (e.g., calcipotriene)
- Phototherapy (light therapy)
- Biologics for severe cases

NCLEX TIP:

Avoid triggers: stress, cold, trauma, infection

Teach: moisturize skin, avoid harsh soaps

2. ECZEMA (ATOPIC DERMATITIS)

Chronic inflammatory skin disorder, often with allergies or asthma history.

CHARACTERISTICS:

- Dry, itchy, inflamed skin
- Common in infants (cheeks) and creases (elbows, knees)

TREATMENT:

- Hydration: thick moisturizers (petroleum-based)

- Antihistamines for itching
- Topical corticosteroids
- Avoid triggers (soaps, allergens)

3. CONTACT DERMATITIS

Localized reaction caused by skin contact with allergen or irritant.

CHARACTERISTICS:

- Red rash, blisters, itching at site of contact
- Common causes: poison ivy, latex, detergents

TREATMENT:

- Remove offending agent
- Topical corticosteroids
- Cool compresses

NCLEX DISTINCTION:

- Atopic = inherited, chronic
- Contact = direct exposure, localized

4. SCABIES

Parasitic skin infestation by the *Sarcoptes scabiei* mite.

CHARACTERISTICS:

- Severe itching (worse at night)
- Burrow lines (tiny thread-like tracks)
- Common between fingers, wrists, waist

TREATMENT:

- Permethrin 5% cream — apply neck down, wash off after 8–14 hrs
- Wash clothing/linens in hot water
- Treat all household members

5. IMPETIGO

Highly contagious bacterial skin infection (usually Staph or Strep)

CHARACTERISTICS:

- Honey-colored crusts around nose/mouth
- Common in kids after minor skin trauma

TREATMENT:

- Topical antibiotics (mupirocin)
- Oral antibiotics if severe
- Good hygiene to prevent spread

NCLEX TIP:

No school until 24 hrs after starting antibiotics

6. STEVENS-JOHNSON SYNDROME (SJS)

A rare but life-threatening skin reaction, often drug-induced (e.g., sulfa drugs, anticonvulsants, NSAIDs)

CHARACTERISTICS:

- Painful red/purple rash, spreads and blisters
- Mucosal involvement (mouth, eyes, genitals)
- Skin sloughs off like a burn

NCLEX PRIORITY:

- Emergency! Stop offending drug immediately
- Treat like burn patient (fluids, infection prevention, pain)

QUICK SUMMARY FOR NCLEX:

Condition	Key Sign	NCLEX Tip
Psoriasis	Silvery scales	Autoimmune, use topical steroids

Condition	Key Sign	NCLEX Tip
Eczema	Dry, itchy skin	Moisturize, avoid triggers
Contact Dermatitis	Local rash	Allergen exposure, remove irritant
Scabies	Night itch, burrows	Permethrin, treat contacts
Impetigo	Honey crust	Contagious! Hygiene key
SJS	Skin peeling + mucosa	Stop drug immediately, emergency care